Northbridge General Hospital (fictional)

100 Example Avenue, Northbridge - fictional address, telephone 000 0000

Discharge Summary

Department of Cardiology

PATIENT	UHID	AGE / SEX
Meera Iyer	**MG25002**	**71 years / Female**
REPORT DATE	REFERENCE NO.	DEPARTMENT
22 Jul 2026	**DSC-33979051**	**Department of Cardiology**

ADMITTED	DISCHARGED	WARD
18 Jul 2026	**22 Jul 2026**	**Cardiac Care Unit, then Ward 4B**

Diagnosis

- Symptomatic paroxysmal atrial fibrillation with rapid ventricular response
- Hypertension
- Reduced renal function (eGFR 41 mL/min/1.73 m2)

Presenting Complaint

Four hours of palpitations with dizziness. On arrival heart rate was 148 /min and irregular, blood pressure 118/70 mmHg, oxygen saturation 97% on room air. ECG confirmed AF with rapid ventricular response. She had taken her anticoagulant that morning.

Hospital Course

She received an intravenous diltiazem bolus and then oral bisoprolol, which was up-titrated from 2.5 mg to 5 mg once daily. She reverted spontaneously to sinus rhythm on day 2. Serial troponin T was below 0.01 ng/mL on two samples, excluding myocardial infarction.

Transthoracic echocardiography showed left ventricular ejection fraction (LVEF) 58 %, mild left atrial dilatation and no significant valve disease.

Day	Date	Heart rate	Blood pressure	Rhythm
1	18 Jul 2026	148 /min	118/70 mmHg	AF, rapid ventricular response
2	19 Jul 2026	92 /min	124/74 mmHg	Sinus rhythm after spontaneous reversion
3	20 Jul 2026	70 /min	130/78 mmHg	Sinus rhythm
4	22 Jul 2026	64 /min	128/76 mmHg	Sinus rhythm; discharged

Echocardiogram Details

- Left ventricular end-diastolic diameter 46 mm
- Left atrial diameter 42 mm (mildly dilated)
- No regional wall motion abnormality
- Aortic and mitral valves: no stenosis, trivial regurgitation
- No left atrial appendage thrombus on transthoracic views
- No pericardial effusion

Nursing Observations (selected)

Date and time	Heart rate	Blood pressure	SpO2	Temperature
18 Jul 2026 14:10	148 /min	118/70 mmHg	97 %	36.9 C
18 Jul 2026 18:00	126 /min	122/72 mmHg	98 %	36.8 C
19 Jul 2026 06:00	92 /min	124/74 mmHg	98 %	36.6 C
19 Jul 2026 14:00	88 /min	126/76 mmHg	98 %	36.7 C
20 Jul 2026 06:00	70 /min	130/78 mmHg	99 %	36.5 C

SYNTHETIC DATA — FOR DEMONSTRATION ONLY

Date and time	Heart rate	Blood pressure	SpO2	Temperature
20 Jul 2026 18:00	68 /min	128/74 mmHg	99 %	36.6 C
21 Jul 2026 06:00	66 /min	126/74 mmHg	98 %	36.5 C
22 Jul 2026 06:00	64 /min	128/76 mmHg	98 %	36.6 C

Multidisciplinary Notes

- Pharmacy: medicines reconciled against the general practice record; no interactions identified with the discharge regimen.
- Physiotherapy: mobilised independently on the ward by day 3.
- Dietitian: low-salt diet advised; no dietary vitamin K restriction is needed with rivaroxaban.
- Nursing: no falls, pressure injuries or bleeding events during the stay.

Patient Education

The plan was discussed with Mrs. Iyer and her daughter. She was shown how to check her pulse, advised to avoid excess caffeine and alcohol, and given written information on atrial fibrillation and on the warning signs of stroke (facial droop, arm weakness, speech difficulty).

Investigations

Test	Result	Reference
Troponin T (0 h and 3 h)	< 0.01 ng/mL, < 0.01 ng/mL	< 0.014 ng/mL
Haemoglobin	12.4 g/dL	12.0 - 15.5 g/dL
Potassium	4.2 mmol/L	3.5 - 5.1 mmol/L
Left ventricular ejection fraction	58 %	>= 55 %

Medications on Discharge

Medicine	Brand	Dose	Frequency	Remarks
Rivaroxaban	Xarelto	15 mg	Once daily with evening meal	Continued; creatinine clearance remains below 50 mL/min.
Bisoprolol	Concor	5 mg	Once daily in the morning	Increased from 2.5 mg during admission.
Amlodipine	Norvasc	10 mg	Once daily	Unchanged.
Ramipril	Tritace	5 mg	Once daily	Unchanged.

Follow-up

- Cardiology clinic in 6 weeks with a 24-hour Holter monitor beforehand.
- GP to check renal function and potassium within 2 weeks.
- Return to the emergency department for palpitations lasting over 30 minutes with chest pain, breathlessness or fainting.

Electronically signed: **Dr. Samuel Ortega, Consultant Cardiologist** on 22 Jul 2026

*** End of report ***

SYNTHETIC DATA — FOR DEMONSTRATION ONLY